A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

29 April 2026

Dr Bee Meng Lee
Premier Health Partners

Dear Dr Lee,

Re: Mrs Amy Fisken
DOB: 02/09/1985 Phone: 0433 855 251
Address: 164 Cashmere Street, TRAVANCORE, VIC, 3032
Email: amycwilliamson@ymail.com
Medicare: 3416441908 / 1

Summary

- **Colonoscopy identifying a small tubular adenoma with low-grade dysplasia.**
- **Surveillance colonoscopy recommended in five years.**
- **Gastroscopy histology identifying a subtle increase in intraepithelial lymphocytes in the duodenum.**
- **Commencement of prucalopride for management of slow-transit constipation.**
- **No action required by you.**

I had the pleasure of conducting a telehealth review with Amy to discuss the results of her recent gastroscopy and colonoscopy.

The colonoscopy was largely unremarkable, with the exception of a single polyp measuring less than 10 millimetres. Histological examination confirmed this to be a tubular adenoma with low-grade dysplasia. This represents a low-risk, precancerous lesion. Given the size and histological subtype, current clinical guidelines dictate a surveillance colonoscopy in five years' time.

The gastroscopy was macroscopically normal; however, duodenal biopsies demonstrated a subtle increase in intraepithelial lymphocytes. Importantly, there was no evidence of villous atrophy or changes to the villous architecture. While this can occasionally be an early histological marker for coeliac disease, it is frequently an incidental finding of no clinical significance. Amy is currently asymptomatic regarding gluten consumption, and we have agreed to manage this conservatively. We will not pursue coeliac serology or repeat gastroscopy at this stage unless she develops symptoms suggestive of malabsorption or gluten intolerance.

We also discussed Amy's chronic constipation, noting that her bowel preparation for the procedure was significantly delayed, which may be suggestive of slow colonic transit. Amy is interested in pharmacological intervention to improve her motility.

Impression and Plan

I have initiated a trial of prucalopride. I have counselled Amy regarding the common side effect of headaches during the first 48 hours of treatment and have advised her to commence with a half-tablet (1 mg) dose before titrating up to a full 2 mg tablet once tolerated. I noted her current use of a high-dose SSRI for

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obsessive-compulsive disorder and reassured her that prucalopride is safe to use in this context.

We will delay further pelvic floor physiotherapy for the time being to assess her response to prucalopride. I have placed Amy on a recall list for a surveillance colonoscopy in five years.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

CC: Ms Isabella Lees-Trinca

Investigation Results:

FISKEN, AMY CLAIRE : SVP 773195

DOB: 02/09/1985 Age: 40 years

164 Cashmere St, Travancore 3032

Results from 11/04/2026 ..... Episode# 26P275615 Test Hist (Histology - Routine)

FINAL ANATOMICAL PATHOLOGY AAnat

Lab #: 2026B009390 SUPPLEMENTARY REPORT H. pylori immunohistochemical stain performed on specimen 2 and is negative. Pathologist: Dr Simin Daneshvar **CLINICAL NOTES** TV polyp. Gas = N. Duo = N. Oesophagus = N. Random colon = N. **MACROSCOPIC DESCRIPTION** 1. Specimen labelled "Oesophagus". A mucosal fragment measuring 6 mm. A1. 2. Specimen labelled "Gastric". Two mucosal fragments measuring 3-5 mm. A1. 3. Specimen labelled "Duodenum". Four mucosal fragments measuring 3-5 mm. A1. 4. Specimen labelled "Transverse Colon Polyp". A tan polypoid fragment measuring 4 x 2 x 2 mm. Processed whole. A1. 5. Specimen labelled "Random Colon". Four mucosal fragments measuring 3-6 mm. A1. **MICROSCOPIC DESCRIPTION** 1. The sections consist of superficial squamous mucosa with no underlying stroma. No significant basal zone hyperplasia is seen and infrequent number of intraepithelial lymphocytes and eosinophils are present. Neutrophils are not seen. Fungal elements are not identified in the H&E stained sections. There is no dysplasia. Glandular mucosa is not included. 2. The sections consist of specialised gastric mucosa with preserved glandular architecture. There are scattered chronic inflammatory cells within the lamina propria. Intestinal metaplasia is not a feature. No neutrophils, Helicobacter-like organisms, or dysplasia identified. 3. The sections consist of small intestinal mucosa with preserved villous architecture. There is a normal complement of chronic inflammatory cells within the lamina propria. The numbers of intraepithelial lymphocytes are mildly increased. No neutrophils, granulomata, parasites or dysplasia are identified. COMMENT: The features are non-specific but can be seen in conditions such as partially treated Coeliac disease, peptic duodenitis, inflammatory bowel disease (Crohn's disease), other autoimmune conditions, drugs, bacterial overgrowth, idiopathic, etc. Correlation with clinical and other laboratory findings is recommended. 4. The sections show colonic mucosa with a tubular architecture. Crypts

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are lined by epithelial cells showing low and moderate grade dysplasia. No high grade dysplasia is identified. 5. The sections show colonic mucosa with preserved crypt architecture. There is a normal complement of chronic inflammatory cells within the lamina propria. No neutrophils, granulomata or dysplasia identified. Features to suggest microscopic colitis are not seen. **PATHOLOGIST'S OPINION 1. Oesophageal biopsy - Superficial squamous mucosa with mild reactive change. 2. Gastric biopsy - Mild chronic inactive gastritis. COMMENT: H.pylori immunostain will be performed and a supplementary report issued when results become available. 3. Duodenal biopsy - Small intestinal mucosa with preserved villous architecture and mild intraepithelial lymphocytosis. Please see above. 4. Transverse colon polyp - Tubular adenoma with low grade dysplasia. 5. Random colon biopsy - Within normal limits.**
Pathologist: Dr Simin Daneshvar

All appointments and enquires
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Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065